

Catheter Care Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To guide practice in the use and management of indwelling catheters.

Scope

- Proposed use, and use of indwelling catheters.

Policy

- Note - This policy is provided as a generic policy and procedure for use when no other more applicable guidelines are available or mandated. Local institutional or regional guidelines, professional guidelines, and statutory body guidelines should, if they exist, be used in preference to this policy.
- Catheter insertion and management will only be performed by staff with the appropriate level of training, skills and authorisation.
- Indwelling catheters will be actively managed in order to maintain the comfort, privacy and dignity, and infection- free status of the Service User, while maintaining staff protection and overall infection control.
- Any Service User requiring, or who may require, an indwelling catheter will be risk assessed and an individual plan of care prepared which is specific to their personal care and medical needs, physical and medical condition, physical environment, and personal wishes.
- Changes of catheter management status, such as initial use, catheter change, issues met and changed management regimes will be communicated to all staff on duty at the time of the change, and the attention of staff drawn to the need to read the Care Plan during the next staff hand-over.
- Before carrying out any operations on catheter systems, perform hand hygiene and don clean gloves, and a plastic apron if local procedure dictates.

Procedure Catheter Insertion

- Refer to current clinical guidelines, such as the Royal Marsden Manual of Clinical Nursing Procedure.

Catheter Maintenance

- Monitor the catheter system at the intervals specified in the Care Plan, checking that tubing and drainage bag are secure and in the proper position to maintain efficient flow while preserving privacy and dignity, and that no blockages are evident.
- Assess urine for clarity, colour, odour or the presence of blood by visually inspecting the urine in the tube, bag or at the sample port or when emptying the drain collection system.
- Maintain a closed drainage system by utilising pre-connected, sealed catheter-tubing junctions.
- Maintain unobstructed urine flow and keep the catheter and collection tube free from kinking.
- Keep the collection bag below the level of the bladder or hips at all times (Note: Exercise particular care to keep bag off the floor).
- Empty the collection bag regularly using a separate, clean collection container for each Service User.
- Replace the collection bag according to the detailed instructions of the system manufacturer and according to training, and of authorised to do so by the Registered Manager. If in doubt ask the Registered Manager to allocate the task to a person with the required skills and authority.
- Provide daily peri-care.
- Perform hand hygiene before and after any contact with the catheter system.

Emptying the Catheter

- Empty the collection system at regular intervals as specified in the plan of care.

- Perform hand hygiene and don clean gloves and a plastic apron.
- Provide a clinical waste disposal container adjacent to the working area.
- Disengage the outlet tube from housing, direct the outlet tube into a collection container, and release the clamp to empty urine from the collection system. Avoid splashing.
- Re-engage the clamp and reinsert the outlet tube into the housing.
- Place gloves and apron in the disposable container and dispose of according to local clinical waste disposal procedures.
- Perform hand hygiene.

Urine Sampling

- This procedure may only be performed by staff with the relevant training and skills, and authorisation in writing by the Registered Manager.
- Request that the procedure be carried out by a suitably skilled and authorised person if in doubt.
- Check the Service User's Care Plan for any indication of issues, including the distress of the Service User, on previous sampling occasions.
- If delivering the sample to the GP surgery for onward transmission to the laboratory, check the latest available deposit time at the surgery and the availability of transport to the surgery. Arrange the transport and plan the sample taking to meet any deadlines.
- Inform the Service User of the proposed sampling, and the reason(s), and reassure them if necessary.
- Perform hand hygiene and don clean gloves and a plastic apron.
- Provide a clinical waste disposal container adjacent to the working area.
- Occlude or kink the drainage tubing a minimum of three inches below the sample port until urine is visible under the access site.
- When urine is visible under the sample port, swab the surface of the sampling port with an antiseptic wipe.
- Using aseptic technique, use a disposable syringe (body only) to slowly draw off a sample through the access port.
- Transfer the specimen into a specimen container, label and place in a transport pouch.
- Replace any clamps moved or removed and unkink all tubes. Check the safe and comfortable operation of the catheter system before any other procedures. Check that the catheter bag and tubes are positioned in such a way as to preserve the privacy and dignity of the Service User.
- Send specimen to the laboratory immediately or refrigerate until transported.
- Place used syringe, gloves and apron in the disposable container and dispose of according to local waste disposal procedures.
- Perform hand hygiene.

Reviewed by Marcus Kerridge McColl
November 2025

Purpose

- To comply with the Data Protection Act 1998 §
- Taking into account:
 - The CCTV Code Of Practice produced by the Information Commissioner;
 - The Human Rights Act 1998;
The Regulation Of Investigatory Powers Act 2000;
Caldicott Report 1997.

Scope

This policy will cover all employees of Crescent Community Care Services Ltd, persons providing a service (voluntary or paid) to the organisation, Service Users, visitors and all other persons whose image(s) may be captured by the system at Crescent Community Care Services Ltd's premises.

Guidance to the Service Provider

This policy is for the guidance of the Service Provider.

Policy

- Crescent Community Care Services Ltd must, before deploying any CCTV system, modifying an existing system, or on first introduction of this Policy, conduct a written impact assessment which must include:
 - An examination of the effects which the deployment has on the privacy of employees of Crescent Community Care Services Ltd; persons providing a service (voluntary or paid) to the organisation; Service Users; visitors and all other persons whose image(s) may be captured by the system.
 - Who will be using the CCTV images? Who will take legal responsibility under the Data Protection Act?
 - What is the organisation's purpose for using CCTV? What are the problems it is meant to address? These could include:
 - Prevention or detection of crime or disorder;
 - Apprehension and prosecution of offenders (including use of images as evidence in criminal proceedings);
 - Interest of public and employee Health and Safety;
 - Protection of public health;
 - Protection of property and assets;
 - To provide evidence in support of misconduct.
 - What are the benefits to be gained from its use?
 - Are the images to be used concurrently i.e. the monitor manned permanently in order to identify and deal with incidents such as unauthorised or dangerous ingress or egress of people, or are images to be made and stored only for review if past incidents are to be reviewed?
 - Can CCTV technology realistically deliver these benefits? Can less privacy-intrusive solutions, such as improved lighting, achieve the same objectives?
 - Do you need images of identifiable individuals, or could the scheme use other images not capable of identifying the individual?
 - Will the particular equipment/system of work being considered deliver the desired benefits now and remain suitable in the future?
 - What future demands may arise for wider use of images and how will you address these?
 - What are the views of those who will be under surveillance?

- What could you do to minimise intrusion for those that may be monitored, particularly if specific concerns have been expressed?
- Where the system will be operated by or on behalf of a public authority, the authority will also need to consider wider human rights issues and in particular the implications of the European Convention on Human Rights, Article 8 (the right to respect for private and family life). This will include:
 - Is the proposed system established on a proper legal basis and operated in accordance with the law?
 - Is it necessary to address a pressing need, such as public safety, crime prevention or national security?
 - Is it justified in the circumstances?
 - Is it proportionate to the problem that it is designed to deal with?

If this is not the case then it would not be appropriate to use CCTV.

- The impact assessment must be written, and stored for future reference, including auditing the system to ensure that it meets the assessed parameters.

Policy application

- Crescent Community Care Services Ltd must designate a Data Controller to take the legal responsibility for the operation and compliance of the CCTV system and resulting data. If the organisation is a legal entity e.g. a limited company, then the Data Controller can be the limited company, it does not need to be a person.

Procedure

Siting the cameras

- It is essential that the location of the equipment be carefully considered, because the way in which images are captured will need to comply with the Data Protection Act.
- All cameras will be located in prominent positions within Service User, public, and staff view and do not infringe on living, circulating (corridors etc), clinical and treatment areas.
- All CCTV surveillance will be automatically recorded and any breach of these Codes of Practice will be detected via controlled access to the system and auditing of the system.
- Signs need to be erected on all entrance points to Crescent Community Care Services Ltd premises and throughout the site to ensure staff and visitors are aware they are entering an area that is covered by CCTV surveillance equipment. The signs must include details on the purpose, organisation and contact details.
- Use of Covert CCTV (Directed) surveillance if required should be considered only with consultation with the Police, whose advice and requirements must be met.

Quality of the images

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- It is important that the images produced by the equipment are as clear as possible in order that they are effective for the purpose(s) for which they are intended. This is why it is essential that the purpose of the scheme be clearly identified. For example, if a system has been installed to prevent and detect crime, then it is essential that the images are adequate for that purpose.
- All camera installations and service contracts should be undertaken by NACOSS approved security companies. Upon installation, all equipment is tested to ensure that only the designated areas are monitored and high quality pictures are available in live and play back mode. All CCTV equipment should be serviced and maintained on an annual basis by a NACOSS approved company.
- The system can consist of cameras recording to digital recorders. These recorders must be located in secure locations, with access control at two levels (a) access to the location available only to authorised persons (b) access to images must be password protected and limited to persons authorised to view them.

Processing the images

- Images, which are not required for the purpose(s) for which the equipment is being used, should not be retained for longer than is necessary. While images are retained, it is essential that their integrity be maintained, whether it is to ensure their evidential value or to protect the rights of people whose images may have been recorded. It is therefore important that access to and security of the images is controlled in accordance with the requirements of the 1998 Act.
- All images that are digitally recorded must be stored securely within the system's hard drives for up to 30 days when they are then automatically erased. Located within the Crescent Community Care Services Ltd's premises there may be sub monitors which display images of public areas, however these must be located within enclosed areas and only accessible to Crescent Community Care Services Ltd 's authorised employees.
- Where the images are required for evidential purposes in legal or disciplinary proceedings, a DVD disc recording is made and placed in a sealed envelope signed and dated and held by the Registered Manager until completion of the investigation. Viewing of images must be controlled by the Data Controller or a person nominated to act on their behalf. Only persons trained in the use of the equipment and authorised by the Data Controller can access data.

Access to and disclosure of images to third parties

- It is important that access to, and disclosure of, the images recorded by CCTV and similar surveillance equipment is restricted and carefully controlled. This will ensure that the rights of individuals are preserved, but also to ensure that the continuity of evidence remains intact should the images be required for evidential purposes e.g. a Police inquiry or an investigation being undertaken as part of the disciplinary procedure.
- Access to the medium on which the images are displayed and recorded must be restricted to such parties and persons as the Data Controller may decide, subject to the restrictions of legislation and good practice.
- Access and disclosure to images is permitted only if it supports the purpose of the scheme. Under these conditions, the CCTV images record book and the appropriate view / release form (see Appendix) must be completed.

Access to images by individuals

- Section 7 of the 1998 Data Protection Act gives any individual the right to request access to CCTV images.
- Individuals who request access to images must be issued an access request form (see appendix). Upon receipt of the completed form, the Registered Manager and Provider's representative will determine whether disclosure is appropriate and whether there is a duty of care to protect the images of any third parties. If the duty of care cannot be discharged then the request can be refused.
- A written response must be made to the individual, giving the decision (and if the request has been refused, giving reasons) within 40 days of receipt of the enquiry. If disclosure is appropriate a payment in advance of £10.00 may be required.

Enforcement

- The Information Commissioner has the power to issue Enforcement Notices where they consider that there has been a breach of one or more of the Data Protection Principles. An Enforcement Notice will set out the remedial action that the Commissioner requires of the Trust to ensure future compliance with the requirements of the Act. The ICO should be notified prior to installation of any CCTV. When informing the ICO you will be required to identify what the data you are recording will be used for.

Reviewed by Marcus Kerridge McColl
December 2025

Challenging Behaviour Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To comply with current best practice in managing challenging behaviour.

Scope

- This policy applies to all staff who have contact with Service Users, and particularly those Service Users who demonstrate challenging behaviour.

Policy

Underpinning Values

- Crescent Community Care Services Ltd adopts the position that challenging behaviour has a purpose in that it meets a particular need for the individual. These needs may be demonstrated through a variety of emotions and behaviours.
- Therefore, it is important that staff can:
 - Develop an understanding of these needs;
 - Anticipate the needs;
 - Soothe situations before they occur;
 - Provide the most appropriate responses to the emotions/behaviours displayed.
- When working with individuals with challenging behaviour staff will be expected to:
 - Recognise the individual's rights to take risks;
 - Ensure that the risk assessment places the Service User central to the purpose;
 - Support the individual through difficulties;
 - Preserve the dignity of the individual;
 - Ensure that rights and choices are promoted;
 - Create an environment in which individuals feel safe and secure;
 - Try to ensure that reflection after an incident places it in the context of the individual's life, and that this can be used to bring about positive outcomes for the individual.

Advance Statements

- Service Users identified as being at risk of disturbed/violent behaviour will be given the opportunity to have their views and wishes recorded, in the form of an advance statement. Advance statements are non-legally binding, but give an opportunity to identify Service Users' preferences about interventions and care.
- Service Users will be encouraged to identify as clearly as possible what interventions they would and would not wish to be used. They must be encouraged to review their wishes with staff, and changes must be clearly documented within Care Plans and other documentation.
- If the professional to whom the wish is being expressed forms the opinion that the Service User lacks capacity to understand the wish they are expressing, the professional must record their opinion, and their reasons for it, alongside the record of the Service User's wish. An advocate or carer should be invited to support a Service User who may lack capacity to understand the wish they are expressing. (Mental Health Act Code of Practice 9.21.)

Service and Profession-Specific Interventions

- A range of interventions, specifically based on the needs of users of particular services, should be provided within the Service to support the prevention and management of violence and aggressive behaviours. These will include the use of specific challenging behaviour models, person-centred Care approaches and speciality programmes in, as appropriate, Learning Disability, Children and Older People Services. Training for those service-specific interventions will be provided.
- In addition, there may be specialist interventions based on professional models, e.g. in clinical psychology. Training would be based within that professional group.

Employee Support

- The Service will ensure, through pre-employment and regular performance appraisal processes, that all employees have the necessary knowledge, skills and physical capability to manage the potential level of violence and aggression in their employment role. Employees will need to be capable of participation in the level of training required for their role.
- All employees who have been involved in an incident of violence or aggression will be offered timely support based upon their individual needs or preferences. Support methods can include:
 - Practical help with transport or accessing medical help;
 - Signposting to specific victim support or individual counselling;

- Critical incident analysis/guided reflection;
- Counselling or psychological therapies;
- Information and assistance in relation to criminal justice procedures;
- Access to Occupational Health Services;
- Training updates;
- Additional training as required;
- Access to staff representation and buddy systems, where appropriate;
- Professional and line management supervision which must be documented.

Record Keeping

- All employees must adhere to the relevant policies, in order to ensure that all information that will promote the proactive prevention and management of challenging behaviour is taken into account and acted upon. These include incident reporting, record keeping and maintenance of contemporaneous multi-professional progress recording.

Definitions

Challenging Behaviours

- "Severely challenging behaviour refers to culturally abnormal behaviour of such intensity, frequency of duration, that the physical safety of the person or others is likely to be placed in jeopardy, or behaviour that is likely to seriously limit or delay access to and use of ordinary community facilities." (Emerson, 1995).
- It is important that the term 'challenging behaviour' is utilised by all services as an all-encompassing concept and not to categorise or diagnose, and therefore assume, specific treatment rationales. For the purpose of the policy challenging behaviour is perceived as being expressed on a continuum from very low level behaviour to severe and extreme behaviours.
- There are a range of definitions of challenging behaviour; Emerson's definition can be used Service-wide.
- To ensure that people with less severely challenging behaviour receive appropriate care and support, Qureshi's (1994) definition is also provided.
- "People are said to display challenging behaviour if they:
 - Had, at some time, caused more than minor injuries to themselves or others, or destroyed their immediate living environment, or;
 - Showed behaviour, at least weekly, that required intervention by more than one member of staff to control or remove them from physical danger, or caused damage which could not be rectified by immediate Care staff or caused at least one hour's disruption, or;
 - Caused more than a few minutes disruption at least daily".
- Behaviours that could be described as challenging include:
 - Physical aggression;
 - Verbal aggression;
 - Self-injury;
 - Property destruction;
 - Non-compliance;
 - Anti-social nuisance behaviour.
- It is accepted that the definition of any given behaviour as challenging is subjective and relative. Therefore it is always necessary to precisely describe the behaviour that is being labelled as challenging in terms of its effects on the person, on their lifestyle, and on other people.

De-escalation

- This is defined as the purposeful use of a complex range of communication and therapeutic intervention skills, based on a knowledgeable understanding of the causes of violence and aggression, which is aimed at the prevention, reduction or management of the probability of violent or aggressive behaviour.
- Refer to the De-escalation Policy and Procedure.

Disengagement Physical Intervention (Breakaway)

- Breakaway interventions are those techniques used in 1:1 defence and disengagement from an attacker in a physical assault.

Restrictive Physical Intervention (Control and Restraint)

- Physical intervention is defined "as any method of responding to challenging behaviour which involves some degree of direct physical force to limit or restrict movement or mobility." (Harris et al, 1996).
- Physical intervention "involves the use of force to control a person's behaviour and can be employed using bodily contact, mechanical devices, or changes to the person's environment." (Department of Health, 2002).
- The purpose of restrictive interventions:
 - "Is to take immediate control of a dangerous situation where there is a real possibility of harm to the person or others if no action is undertaken. And
 - End or reduce significantly the danger to the patient or others". (Mental Health Act 1983 Code of Practice 2025 26.36.)
- Refer to Restraint Policy and Procedure.

Mechanical Restraint

- This is defined as the use of splints, arm cuffs, weighted blankets or any other device that is used to prevent injury by restricting movement or mobility which has been considered within an individual's risk during physical interventions; this would be detailed in the plan of Care for individuals. Protocols and training issues regarding the use of mechanical devices in physical interventions would be detailed in the relevant service areas documentation and policy.
- Refer to Restraint Policy and Procedure.

Consent

- "Consent" is a Service User's agreement for a Care or health professional to provide Care. Service Users may indicate consent non-verbally (for example, by presenting their arm for their pulse to be taken), orally, or in writing. For the consent to be valid, the Service User must:
 - Be competent to take the particular decision;
 - Have received sufficient information to take it;
 - Not be acting under duress.

Capacity

- Having mental capacity means that a person is able to make their own decisions. The Mental Capacity Act 2005 is designed to cover situations whereby someone is unable to make decisions because of an impairment of, or a disturbance in the functioning of, their mind or brain. The Act says that a person is unable to make a particular decision if, due to an impairment of, or a disturbance in the functioning of, their mind or brain they cannot:
 - Understand information about the decision to be made (the relevant information);
 - Retain the information in their mind;
 - Use or weigh that information as part of the decision-making process;
 - Communicate their decision.
- See Paragraph 4.14 Mental Capacity Act 2005 Code of Practice.

Care Interventions

- A proactive recovery-focused approach aimed at preventing the likelihood of challenging behaviour occurring. The gathering of information from Service Users, their carers and families, colleagues and other agencies will lead to a formulation of the risk issues.
- That assessment will identify preventative strategies, both to promote a positive environment and to minimise the risks. Specific management guidelines will be based on individual planning of Care.

- Interventions may include the promotion of alternative coping skills and activities, environmental change, de-escalation, use of rapid tranquilisation, physical control and restraint, and seclusion.
- Minimum level of interventions must be used and Service Users supported in developing their own positive coping and risk management skills. Exceptions to these principles may be requested through an individual's advance request. Further functional analysis of behaviours, near misses and incidents must be undertaken to inform the clinical and management strategies.

PROCEDURE

Incident Reporting

- All incidents of challenging behaviour, violence and aggression, physical or non-physical, and near misses to incidents will be reported using the Record of Challenging Behaviour form in this document, or the HS01 Accident and Incident Reporting Policy and Procedure - where appropriate.
- Employees will be supported in the incident reporting procedure and advised to complete requests as soon after the incident or near miss as possible.

Post Incident Review

- The purpose of a post-incident debrief is "to identify and address physical harm to Service Users or staff, ongoing risks and the emotional impact on Service Users and staff, including witnesses" and to "determine the factors that contributed to an incident that led to a restrictive intervention, identify any factors that can be addressed quickly to reduce the likelihood of a further incident and amend risk and care plans accordingly." (National Institute for Health and Care Excellence: NG10, *Violence and Aggression: Short-term management in Mental Health, Health and Community settings 2025*.)
- "The aim of a post incident review will be to seek to learn lessons, support staff and Service Users and encourage the therapeutic relationship between staff, Service Users and their carers." (NICE 2005).
- A post incident review will take place as soon as possible and within at least 72 hours of an incident ending. If possible, a person not directly involved in the incident will lead the review. The review will address:
 - What happened during the incident;
 - Any trigger factors;
 - Each person's role in the incident;
 - Their feelings at the time of the incident, at the review and how they may feel in the near future;
 - What can be done to address their concerns.
- A documented record of this must be drawn up, especially in relation to restraint. If a Service User is restrained, this must be discussed and the reasons why this way deemed necessary shared. Collaborative planning to prevent recurrence is essential.
- Service Users and visitors who witness restraint must also be given an explanation of the need for forceful intervention to ensure understanding and allow any anxieties or fears to be voiced.

Lone Working

- Many mental health and learning disability employees work as lone practitioners either in Service premises or in community settings. The systems and procedures to be used to prevent, minimise and manage risk for lone workers are outlined in the lone working policies.

Contacting the Police in the Management of Violence and Aggression

- All Service employees are advised to contact the local police force, using the emergency 999 call number, if they are involved in a challenging behaviour or violent or aggressive situation that cannot be de-escalated or managed by the range of clinical procedures and interventions available to clinical practitioners.

This is particularly important if a violent incident is provoked or initiated by those who are not in receipt of clinical services and are not known to the clinical practitioner.

Engagement and Observation

Engagement and observation are therapeutic risk management interventions that can be utilised to support the management of challenging behaviour.

Rapid Tranquilisation

There is a range of interventions where medicines are used to assist in the prevention, reduction and management of violence and aggression. Specific use of prescribed medication to ensure rapid tranquilisation of Service Users who are becoming increasingly disturbed, violent or aggressive, or where their challenging behaviours are escalating, may be used according to individual and pre-prescribed principles. Specific training is mandatory for employees to use these approaches.

Primary Prevention

When used as a primary prevention, medication must only be used where a Service User has a mental health or co-morbid mental health problem that is alleviated by medication. A physical health and pharmacology assessment will be completed prior to administration of as required medication, and before the use of a restrictive physical intervention.

Secondary Prevention Reaction

When considering the use of medication as a secondary prevention, or reactive strategy, the Service advocates evidence-based approaches less dependent on medication and requires compliance with the guidance given in the Guidance for Restrictive Physical Interventions (Department of Health, 2002, p18).

De-escalation Techniques

- NICE guidelines indicate that health and social care provider organisations should give staff training in de-escalation that enables them to: recognise the early signs of aggression and be able to respond appropriately. These guidelines also stress the general principle that staff should:
 - Establish a close working relationship with Service Users that will allow them to be aware of changes in mood. (National Institute for Health and Care Excellence: NG10, *Violence and Aggression: Short-term management in Mental Health, Health and Community settings* 2025.)
 - Refer to De-escalation Policy and Procedure.

Disengagement Physical Intervention (Breakaway)

- The Service supports the use of Breakaway interventions that are taught in order to enable the staff to disengage from an attacker in a physical assault. The Service acknowledges that the use of reasonable force may also be necessary. Wherever possible, and from the formulation derived from the clinical risk assessment, a plan of Care prescribing the use of such interventions, or other preventative strategies, would be compiled.

Reasonable Force

- There is no legal definition of 'reasonable force'. It is therefore not possible to comprehensively set out when it is reasonable to use force, or the degree of force that may reasonably be used. It will always depend upon the circumstances of the case.
- There are two relevant considerations:
 - The use of force can be regarded as reasonable only if the circumstances of the particular incident warrant it. The use of any degree of force is unlawful if the particular circumstances do not warrant the use of physical force. Therefore physical force could not be justified to prevent an individual from committing a trivial misdemeanour, or in a situation that clearly could be resolved without force.
 - The degree of force employed must be in proportion to the circumstances of the incident and seriousness of the behaviour or the consequences it is intended to prevent. Any force used must always be the minimum needed to achieve the desired result.
- Reasonable force may be used for Service Users who are detained under the Mental Health Act, where it is "therapeutically necessary" to be medicated, restrained, fed or secluded. (See Section 63, part 4 of the Mental Health Act 1983 concerning medical treatment for mental disorder not requiring the consent of the patient.) For medical treatment for physical disorders, this cannot be administered without the consent of the person. For someone who



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lacks the mental capacity to consent, reasonable force may be used where the treatment is in the person's best interest. (Section 6 of the Mental Capacity Act 2005.)

Restrictive Physical interventions

- When violent, aggressive or challenging behaviour cannot be de-escalated and it is assessed that there are significant risks to the safety and security of the perpetrator and/or the potential victim(s), employees may need to use restrictive physical interventions (control and restraint). Those using these interventions must demonstrate competency through assessment.
- The training for the use of physical intervention is generic, but specific interventions for specialist needs can be identified for employees during training or from requested consultation.
- For the purpose of this policy, physical intervention, as defined, does not involve the use of mechanical devices. The Service supports the use of restrictive physical intervention techniques that are taught to enable staff to respond to challenging behaviour using direct physical contact to limit or restrict the movements of another, or disengage as a last resort when a Service User is presenting a high risk of harm to themselves or others (see Protocol for Good Practice in Restrictive Physical Intervention).

Deprivation of Liberty

- Where restrictions or restraint on Service Users would constitute a deprivation of the person's liberty, this would be in contravention of the person's human rights, unless authorisation for the deprivation of liberty was sought. For further guidance on this, reference should be made to the QCS Policy and Procedure on the Deprivation of Liberty Safeguards.

IMPLEMENTATION

- Awareness of this policy will also be disseminated to all Service employees through a line management briefing, cascaded through the operational and corporate service structures, supported by a standardised briefing.
- The Registered Manager will designate a person to assess the environmental risks in premises within their areas of responsibility.
- An overall service specific risk assessment will be completed with the involvement of all staff in order to determine the level of risks posed, and the training requirements necessary to manage those risks.
- The outcome of the risk assessments will be assessed, reviewed and agreed through the service specific governance systems.
- A register of environmental risks is to be maintained.
- Mandatory training will be provided in line with the organisational training needs analysis, with attendance monitored via human resources and reports sent to local managers to ensure mandatory training needs are met.
- For training that the Service has not classified as mandatory, managers should consider the necessary competencies for the specific work required and arrange for these needs to be met through the appraisal and personal development plan process. Access to support to meet these needs is outlined in the training policy.
- Employees will be expected to complete a self-assessment of physical health in relation to undertaking training, rehearsal and practice in the delivery of breakaway and physical interventions. Any concerns regarding physical capability will be referred to the Registered Manager.

Training Levels

- The Service will ensure that the appropriate training and education is available to implement this policy. All employees will be expected to participate in the training and education programmes as determined by the organisational training needs analysis and identified as required by their role. Their competencies will be monitored through the Service appraisal, supervision systems and Human Resources Systems. Appropriate recording will be maintained in staff's personal files.
- Different forms of training are available:

- **Theory** - Following the Safety, Management and Security standard national curriculum on the introduction to violent and aggressive behaviour, as well as promoting personal safety competency outcomes of applied knowledge, as assessed by an appraiser in a work setting.
- **De-escalation and Breakaway training** - All of the content of the theory component, plus positive communication and the use of functional analysis in the prevention and management of violence and aggression, de-escalation interventions, preventative and breakaway physical interventions. Competency outcomes will be assessed during training.
- **Bespoke Breakaway training** - All of the content of the theory component, plus positive communication and use of functional analysis in the prevention and management of violence and aggression, De-escalation interventions, preventative and breakaway physical interventions including a seated restraint technique. Competency outcomes are assessed during training.
- **Physical intervention** - All the content of the theory component and de-escalation and breakaway, plus physical interventions used in the management of violence and aggression and positional asphyxia. Competency outcomes are assessed during training.

PROTOCOL FOR CHALLENGING BEHAVIOUR ASSESSMENT AND MANAGEMENT

- This protocol outlines specific practice underpinning the Care of those designated as having challenging behaviours.
- For each individual:
 - An assessment for capacity to consent must be carried out, and if capacity is present, permission must be gained for any assessment or intervention prior to it being carried out. If they are deemed to lack capacity to consent, or they are refusing treatment, professionals are to follow the 'best interest' rule.
 - A comprehensive assessment and formulation must be completed, incorporating:
 - Comprehensive assessment;
 - A risk assessment tool;
 - Functional assessment.
 - All interventions (including physical interventions) must be carried out and documented.
 - Other interventions must match well with the assessment.
 - The Service User must have been involved in the development of the intervention plan.
 - Primary and Secondary preventions and agreed Reactive strategies (including any restrictive physical interventions) must be written into the Service User's Care plan.
 - Any restrictive physical interventions must be carried out in accordance with the current Guidance for Restrictive Physical Interventions from the National Institute for Health and Care Excellence, General Services Association and the British Institute for Learning Disabilities.
 - Following any incident involving challenging behaviour (excluding those behaviours which are pervasive, in occurrence more than twice daily and do not cause significant physical harm), the following must be carried out:
 - A post-incident record must be completed on the case notes;
 - A post-incident analysis must be carried out and any recommendations carried out (e.g. amendments to Care Plan);
 - Where appropriate, a debrief must be carried out with the person who has presented the challenging behaviour to check their physical, mental and emotional well-being;
 - A debrief must be given to any staff members involved in the incident and a range of resources for support will be offered.

GOOD PRACTICE IN RESTRICTIVE PHYSICAL INTERVENTIONS

Good Practices During Restraint

Introduction

- Restraint techniques must only be used as a last resort, when all primary and secondary interventions have failed and the level of risk is such that more intensive interventions are required in order to maintain safety. Service Users with a history of challenging behaviour/violence and aggression must, as soon as practicable, have a relevant care/treatment plan drawn up, in collaboration with the multi-disciplinary team, Service User, carers, family, and/or



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advocates. Where possible this must describe and set out a broader strategy for addressing the Service User's needs and challenging behaviour.

- Gender, age, size and build, and in addition physical and sensory impairment issues, must be taken into account (e.g. pregnancy, children, younger persons and older persons).
- Wherever possible the Service User must be approached and an agreement sought to stop the behaviour, or co-operate with a request. Before any attempt is made to restrain an individual a visual check must be made for weapons. If there is any doubt, the Service User must not be approached and police summoned to assist. If weapons are visible then the Service User must be asked to put them down in a neutral area and not to hand them over.

Responsibilities

- The person controlling the individual's head will assume control of the intervention and will manage the situation. Other individuals, present at the situation, will take direction from the head person. Compliance with this procedure ensures minimisation of risk to staff and Service Users.
- There must be continual verbal de-escalation between the individual in control of the incident/situation, and the Service User. Communication between the rest of the team must be minimal.
- Staff will demonstrate at all times respect dignity and privacy and ensure that there is communication between the Service User and themselves.
- During any physical restraint intervention the Service User's physical condition must be monitored in order to reduce physical risks e.g. positional asphyxia. Where mechanical restraint is employed during and after physical intervention, guidance in the Mental Health Act Code of Practice says this should be regularly reviewed by a registered medical practitioner. This policy should be read in conjunction with Chapter 26 of the Code of Practice titled 'Safe and Therapeutic responses to disturbed behaviour'.
- When possible, other Service Users and staff not involved with restraint must be relocated to another area.
- Restraint must be used as a last resort. At the earliest opportunity the Service User will be disengaged from the restrictive physical intervention when it is deemed safe and practicable to do so.
- The Service does not support the deliberate application of pain being applied to Service Users to gain compliance. It is acknowledged "that there may, however, be high risk situations where an element of pain - for both the individual and staff - may be unavoidable if the emergency is to be safely resolved". (Guidance from the National Control and Restraint General Services Association at <http://ncrgsa.com>.)
- The Service training needs analysis describes who must attend which form of Management of Violence and Aggression training, and be deemed to be competent before engaging in these practices.
- Where physical restraint is used, staff must record the decision and the reasons for it and document and review every episode of physical restraint, which must include a detailed account of the restraint and record the post- incident review.
- Staff must use debriefing and post-incident review following all episodes of restrictive physical interventions.

Reviewed by Marcus Kerridge McColl
November 2025

Changes in Health State Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To respond appropriately to changes in the Health Status of Service Users

Scope

- All Service users

Policy

- Staff are vigilant to the day to day good health of Service users and promptly report any changes or concerns. Service Users are assisted to seek help and advice from their GP when feeling unwell, or staff ensure any health concern is assessed as soon as possible by a medical practitioner
- Information on the Health status of the Service User and details of any known health issues and contact arrangements in the event of illness are recorded in the Service user Guide and also on the computer system in the office

Procedure

- Care Plans show a thorough assessment of the Health issues of the Service user and a current Health Action Plan
- Details of any signs and symptoms that may denote that a Service user is unwell are given in Care Plans
- Any drug reactions / interactions are noted in the Care Plans and medicine administration records along with the contradictions of medicines
- Service Users are helped to express their state of health and encouraged to communicate in an appropriate way any pain or ache or discomfort. This might include the use of a sign, gesture, symbol or object of reference for example.
- The Use of analgesia is clearly defined in the care plan. Staff should observe any instruction for administration, including method and timing ensuring that combined medicines (particularly those containing paracetamol) are carefully managed. This may include cough medicines, cold powders and similar products.
- In the event of an unplanned medical appointment staff will inform relevant family members or emergency contacts.
- Any health changes care changes or medical changes need to be reported to the team and relevant care plans updated

Reviewed Marcus Kerridge-McColl – November 2025

Changes to Contracted Hours Policy and Procedure Crescent Community Care Services Limited

Purpose

- To ensure that staff changes are managed sufficiently as to not affect the Service to the Clients of Crescent Community Care Services Limited

Scope

- To all staff members of Crescent Community Care Services Limited

Policy

- To ensure that there is adequate cover for all calls to clients of Crescent Community Care Services Limited
- To maintain staffing levels at all times

Procedure

- All staff members are to give at least 2 weeks notice for permanent changes to their contracted working hours. This is to ensure that the changes can be planned effectively and the Service users calls a managed efficiently.

Reviewed Marcus Kerridge-McColl
November 2025

Child Protection Policy & Procedure

Crescent Community Care Services Limited

Purpose

- To comply with good practice in child protection in situations where employees may have unsupervised access to children while working, for instance where a child lives in a Service User's own home.
- To comply with safe practice where children visit Service Users in registered premises, or in the Service User's own home.

Scope

- Situations whereby employees have unsupervised access to children while working.
- Situations where children visit Service Users in registered premises, or in the Service User's own home.

Policy

- We at Crescent Community Care Services limited are committed to practices which protect children from harm. Staff in this organisation accept and recognise our responsibility to develop awareness of issues that can cause children and young people harm.
- We will endeavour to safeguard children and young people by:
 - Adopting child protection guidelines through a code of behaviour for staff and volunteers;
 - Sharing information about child protection and good practice with children, parents and staff;
 - Sharing information about concerns with agencies who need to know, and involving parents and children appropriately;
 - Following carefully the procedures for recruitment and selection of staff;
 - Providing effective management for staff through supervision, support and training;
 - Providing a safe environment for children.

Procedure

CODE OF BEHAVIOUR

Statement of Intent



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- It is the policy of Crescent Community Care Services Limited to safeguard the welfare of all children and young people by protecting them from physical danger, and all forms of abuse, including physical, emotional and sexual harm.
- Crescent Community Care Services Limited is committed to creating an environment in which young people can feel safe and comfortable and secure while in contact with any of Crescent Community Care Services Limited's employees. Personnel should at all times show respect and understanding for individuals' rights, safety and welfare, and conduct themselves in a way that reflects the ethos and principles of Crescent Community Care Services Limited.

Guidelines for all Crescent Community Care Services Limited employees

Attitudes

- Staff should be committed to:
 - Treating children and young people with respect and dignity;
 - Always listening to what a child or young person is saying;
 - Valuing each child and young person;
 - Recognising the unique contribution each individual can make;
 - Encouraging and praising each child or young person.

By Example

- Staff should endeavour to:
 - Provide an example, which we would wish others to follow;
 - Use appropriate language with children and young people and challenge any inappropriate language used by a young person or child, or an adult working with young people;
 - Respect a young person's right to privacy.

One-to-one Contact

- Staff should not spend excessive amounts of time alone with children, away from others.
- In the unlikely event of having to meet with an individual child or young person staff should make every effort to keep this meeting as open as possible.
- If privacy is needed, ensure that other staff are informed of the meeting and its whereabouts.

Physical Contact

- Staff should never:
 - Engage in sexually provocative or rough physical games, including horseplay;
 - Do things of a personal nature for a child or a young person that they can do for themselves. If such an incident arises, for example, where a child or young person has limited mobility Crescent Community Care Services limited staff should seek a member of their family to deal with such an incident;
 - Allow, or engage in, inappropriate touching of any kind.

General

- Staff should:
 - Be aware that someone might misinterpret our actions no matter how well intentioned;
 - Never draw any conclusions about others without checking the facts;

- Never allow themselves to be drawn into inappropriate attention-seeking situations such as tantrums or crushes;
- Never exaggerate or trivialise child abuse issues or make suggestive remarks or gestures about such issues, especially to a child or young person, even in jest.

Risk Assessments

- Where children are admitted to a premises, or are resident on the premises, all risk assessments carried out on the environment, equipment, substances and anything with which the child may come into contact must specifically assess the risks to and from children. Until this range has been checked, extended where necessary, and all advised actions carried out, children should not be admitted on the premises.

SHARING INFORMATION ABOUT CHILD PROTECTION AND GOOD PRACTICE WITH CHILDREN, STAFF AND VOLUNTEERS

- Good communication is essential in any organisation. In Crescent Community Care Services Limited, every effort will be made to ensure that, should individuals have concerns, they will be listened to and taken seriously.
- It is the responsibility of the management to ensure that information is available to, and exchanged between, all those involved in this organisation and its activities. Some information is confidential and should only be shared on a strictly need-to-know basis.

Children and Young People

- Children and young people have a right to information, especially any information that could make life better and safer for them. Crescent Community Care Services Limited will act to ensure they have information about how, and with whom, they can share their concerns, complaints and anxieties.
- When sharing information, Crescent Community Care Services Limited personnel will be sensitive to the level of understanding and maturity, as well as to the level of responsibility, of the people with whom they are sharing.

Parents

- Parents / persons with parental responsibility are ultimately responsible for their children's welfare at all times, and they should be assured that their children are in contact with staff employed by a credible organisation.
- Where the children are not the recipient of the Service, but are simply visiting a Service User Parents should also closely supervise at all times and accept responsibility for their children when they are visiting a Service User at home, whether that is the Service User's own home or a registered premises such as a hospital or care home. It is the responsibility of the parent(s) to ensure that their children:
 - Are made aware of the need for the privacy of the Service Users;
 - Are made aware of the need to not disturb other Service Users in the vicinity;
 - Are made aware that a registered premises, and often a Service User's own home, may hold materials which may be harmful to children. The parents must be vigilant for such dangers, and children aware that they should not touch or consume anything without checking with their parents first;
 - Are made aware that a registered premises, and often a Service User's own home, may contain equipment which may be harmful to children. The parents must be vigilant for such dangers, and children aware that they should not touch anything without checking with their parents first.

Staff

- As an organisation whose employees may occasionally be in contact with children and young people in the workplace, it is imperative that each staff member of the Crescent Community Care Services Limited staff in this situation is aware of their responsibilities under the Child Protection legislation and has a working knowledge of The companies procedures.
- Staff will report to their line manager if children or young people enter their workplace.

- The line manager will ensure that all staff who do or may have contact with children or young people receive direct supervision with regard to the contents of this policy, and conduct regular checks on the workplace.

Other Bodies

- A copy of our Child Protection Policy will be made available to any other appropriate body.

SHARING INFORMATION ABOUT CONCERNS WITH AGENCIES WHO NEED TO KNOW, AND INVOLVING PARENTS AND CHILDREN APPROPRIATELY - PROCEDURE FOR REPORTING ALLEGATIONS OR SUSPICIONS OF ABUSE

- In any case where an allegation is made, or someone in Crescent Community Care Services Limited has concerns, a record should be made. As far as is practically possible, details must include:
 - Name of child or young person;
 - Age;
 - The Agency address (if known);
 - Date of birth (if known);
 - Name(s) and address(es) of parent(s), or person(s) with parental responsibility;
 - Telephone numbers if available;
 - Is the person making the report expressing their own concerns, or passing on those of somebody else? If so, record details;
 - What has prompted the concerns;
 - Include dates and times of any specific incidents;
 - Whether the child or young person has been spoken to;
 - If so, what was said;
 - Whether anybody has been alleged to be the abuser;
 - If so, record details;
 - Who has this been passed on to, in order that appropriate action is taken? e.g. school, designated officer, social services etc;
 - Whether anyone else has been consulted? If so, record details;
 - **ACTION TAKEN.**

Record Keeping

- All records, information and confidential notes should be kept in separate files in a locked drawer or filing cabinet.
- Only designated persons will have access to these files.

Disclosure

- Never guarantee absolute confidentiality, as Child Protection will always have precedence over any other issues.
- Listen to the child, rather than question him or her directly.
- Offer him / her reassurance without making promises, and take what the child says seriously.
- Allow the child to speak without interruption.
- Accept what is said – it is not your role to investigate or question.

- Do not overreact.
- Alleviate feelings of guilt and isolation, while passing no judgement.
- Advise that you will try to offer support, but that you must pass the information on.
- Explain what you have to do and whom you have to tell.
- Record the discussion accurately, as soon as possible after the event.
- Use the child's words or explanations – do not translate into your own words, in case you have misconstrued what the child was trying to say.
- Contact one of the Crescent Community Care Services Limited's Directors or Registered Manager for advice / guidance.
- The Registered Manager may then discuss the concern / suspicion with the relevant organisation, and, if appropriate, make a direct referral.
- If either Registered Manager is not available, or it is inappropriate to approach them, the member of staff with the concern should make direct contact with the relevant organisation themselves.
- Record any discussions or actions taken within 24 hours.

Reviewed Marcus Kerridge-McColl
September 2025

Choice of Carer Gender Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To support the choice of the Service User.

Scope

- All Service Users.

Policy

- Service Users will be asked to express any preference they have for a Carer of a particular gender.
- Where it is not possible for the Service User to indicate a choice, family preferences will be followed.
- Where it is not possible for the Service User to indicate a choice, and there is no family or other relevant advocate, Care will always be provided by carers of the same gender as the Service User.

Procedure

- The Service User, family member, or relevant advocate will be asked if they have a preference for the gender of the carer.
- Their choice will be recorded on the Service User Care Plan.
- Staff of the appropriate gender will be arranged according to identified Service User needs.
- Where a male Carer is assigned to provide personal Care for a female Service User, the Service User will be offered a female chaperone.

Reviewed by Marcus Kerridge McColl

November 2025

Clinical Waste disposal Policy & Procedure
Crescent Community Care Services Limited

Purpose

- To establish criteria for the handling and disposal of waste products resulting from Crescent Community Care Services Ltd activities, as well as a set of procedures to guide staff in terms of legislation and best practice.

Scope

- All employees.

Policy

- **Crescent Community Care Services Ltd** is committed to the protection of the environment through the implementation of an effective waste management programme that meets or exceeds all legislative and regulatory requirements placed upon it.
- **Crescent Community Care Services Ltd** will comply with:
 - The Controlled Waste Regulations;
 - The Environmental Protection Act including the Duty of Care Regulations;
 - The Carriage of Dangerous Goods Regulations;
 - The Hazardous Waste Regulations.
- Principles for consignments of clinical waste:
 - It is **Crescent Community Care Services Ltd** policy to dispose of normal clinical waste in sealed bags to a registered clinical waste disposal contractor, provided it does not contain pharmaceutical products.
 - Alternative disposal methods may be considered where the producer confirms that no such waste needs to be incinerated, which is confirmed by regular local audits.
 - Where the waste is known to be non-infectious, then it may be consigned as such. Otherwise it should be consigned as hazardous by infection; even if only very small amounts of infectious materials are (or may be) mixed with otherwise non-infectious waste. Any waste contaminated with, or containing cytotoxic or cytostatic medicines must be consigned separately to the above.
 - Controlled drugs are to be denatured prior to disposal.
 - Waste sharps must be placed in an approved container and ideally separately consigned or added to and consigned with the cytotoxic waste.
 - Materials which are not hazardous and therefore do not need to meet the clinical waste disposal regulations should not be disposed of in hygiene bags. Disposal of clinical waste is very costly compared to non-hazardous waste disposal.

Procedure

- **Duty of Care Regulations:**
 - The Duty of Care Regulations (See Appendix attached to this policy) must be brought to the attention of all employees who are producing, handling, or storing all types of controlled waste. Breach of the Duty of Care Regulations is a criminal offence.



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- The waste producer (Crescent Community Care Services Ltd) is responsible for complying with the clinical waste disposal regulations. This responsibility cannot be assumed to be taken up by a third party, such as the waste removal contractor.
- Therefore, the waste producer must at all times satisfy themselves that the waste disposal contractor is meeting the regulations on the producer's behalf.
- **Clinical waste disposal procedure:**
 - The procedure to be used for the disposal of clinical waste is as follows:
 - Waste is accurately identified as being "clinical";
 - Waste is appropriately packaged by type;
 - Waste is stored correctly;
 - The waste is stored securely prior to disposal;
 - Clinical waste transfer note has been completed.
- **Identification of clinical waste:**
 - Responsibility for identifying clinical waste lies strictly with the producer. It is important to remember for identification purposes that all waste contaminated with clinical waste is considered to be clinical waste itself.
- **Materials identified as clinical waste must be disposed of through a licensed contractor:**
 - Under no circumstances must clinical waste be allowed to enter the general waste stream.
 - Clinical waste must be carefully segregated from other waste whilst in production and storage, and must never be placed in internal or external general waste bins.
- **Storage and packaging of Clinical waste:**
 - Under the Duty of Care Regulations (section 34 Environmental Protection Act 1990), all waste must be kept safe from:
 - Corrosion and wear of waste containers;
 - Accidental spilling or leaking;
 - Accident or weather breaking contained waste open and allowing it to escape;
 - Waste blowing away whilst stored or transported, scavenging of waste by vandals, thieves, children, trespassers or animals.
- **Types of Packaging:**
 - Requirements for packaging vary depending on, for example, the category of the waste, whether it is transported off-site, and the method of final disposal.

Waste Group	Type of Clinical Waste
Group A	Includes the following items: identifiable human tissue*, blood animal carcasses and tissue from veterinary centres, hospitals, or laboratories. Soiled surgical dressings, swabs and other similar soiled waste. Waste materials, for example from infectious disease cases, excluding any in groups B-E.
Group B	Discarded syringe needles, cartridges, broken glass and other contaminated disposable sharp instruments or items.

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Group C	Microbiological cultures and potentially infected waste from pathology departments and other clinical or research laboratories.
Group D	Drugs or other pharmaceutical products.
Group E	Items used to dispose of urine, faeces and other bodily fluid secretions or excretions, which do not fall within group A. This includes used disposal bed pans or bed pan liners, incontinence pads, stoma bags and urine containers.

Group A:

- Group A clinical waste (waste that is contaminated with body fluids, human and animal tissue) should be placed in designated bins lined with yellow clinical waste sacks only. The sacks need to comply with the requirements of CDGRR[1] / ADR. This means that they need to be design-tested and certified (usually referred to as UN type-approved) for transportation off site.
- Containers should be no more than three-quarters filled or a maximum of 4KG weight securely tied (not stapled).
- For ease of storage and handling, it is advised that tissue waste is kept packaged in clinical waste sacks frozen in a dedicated freezer.

Group B:

- Clinical waste sharps should only be stored in sharps containers that comply with British standard BS 7320 and UN 3291.
- When three-quarters full, the container should be securely closed – on no account should attempts be made to reopen a sealed sharps container.
- Sharps containers should not be placed in yellow clinical waste sacks, but kept separate for collection.

Group C:

- Any group C waste which is not sterilised before leaving site has to be packaged in accordance with approved requirements (i.e. UN type-approved container).

Group D:

- Group D clinical waste must be transported in appropriate UN type-approved packaging. The use of clinical waste sacks is not acceptable.

Group E:

- Group E wastes are not considered dangerous for transport by road under CDGRR and do not have to be in UN type-approved containers. However, risk assessments under CoSHH may indicate the need for precautions to ensure the health and safety of people handling the waste. Producers need to take this, and the possibility of causing offence, into account when making decisions about packaging.

Labelling:

- Packaging for clinical waste needs to be labelled to show clearly the type of waste. If the waste is transported off-site for final disposal, labels must meet the requirements of CDGRR. These regulations prohibit consigning the waste for transport unless the following information is clearly shown:
 - The appropriate designation of the waste. For example, the designation of unspecified infectious waste is "CLINICAL WASTE, UNSPECIFIED, N.O.S";
 - The appropriate UN Number. In the case of CLINICAL WASTE UNSPECIFIED, N.O.S this would be UN 3921 (The number must always be preceded by the letters UN;
 - The appropriate danger sign. The danger sign for infectious substances, which is needed for waste classified as UN 3921, is shown below.



Dealing with accidents incidents and spillages:

- Employers at all points of the waste chain need written procedures for dealing with accidents or incidents, which address:
- Immediate first aid measures;
- Immediate reporting to a responsible, designated person;
- Recording of the accident/incident;
- Investigating the incident and implementing remedial action. Initial investigation should preferably take place before any damaged container is removed;
- Retaining, if possible, the item and information about its source to help identify possible infection risks;
- Attendance by any injured person to an A&E department and, if an employee, with subsequent referral to a General Practitioner as soon as possible;
- In the cases of sharps injuries, procedures also need to cover arrangements for suitable medical advice and counselling.

Spillages:

- Clear written procedures need to be in place specifying the following reporting and investigation procedures:
 - Use of a safe system of work for clearing up clinical waste;
 - Appropriate requirements for decontamination;
 - Protective clothing.
- Spillage kits should contain the following:
 - Disposable gloves;
 - A disposable apron;
 - Clinical waste sack;
 - Paper towels;
 - Disposable cloths;
 - Disinfectant recommended (by, for example, local control);
 - Means of collecting sharps.

Spillage Kits:

- Disinfectants containing 10,000-ppm available chlorine are recommended for spillages. The use of sodium dichloroisocyanurate (NaDCC) granules is also generally recommended because made-up solutions lose activity with time and require regular replacement. Suitable inert, absorbent materials may be used to deal with liquid spillages after disinfecting.
- The use of certain disinfectants themselves may present a health risk in confined spaces. COSHH applies to such risks and appropriate precautions must be specified based on risk assessments. Glutaraldehyde must not be used.
- Appropriate equipment needs to be provided for collecting spilled waste and placing it in new containers. Sharps must not be picked up by hand. Spilled waste and any absorbent materials must be placed in a clinical waste container for disposal.

Appendix - Statutory Instrument 1991 No. 2839

The Environmental Protection (Duty of Care) Regulations 1991

STATUTORY INSTRUMENTS

1991 No. 2839

ENVIRONMENTAL PROTECTION

The Environmental Protection (Duty of Care) Regulations 1991

Made: 16th December 1991

Laid before Parliament 17th December 1991

Coming into force 1st April 1992: 1st April 1992

The Secretary of State for the Environment as respects England, the Secretary of State for Wales as respects Wales and the Secretary of State for Scotland as respects Scotland, in exercise of the powers conferred on them by section 34(5) of the Environmental Protection Act 1990[1] and of all other powers enabling them in that behalf, hereby make the following Regulations:

Citation, commencement and interpretation

1. -

(1) These Regulations may be cited as the Environmental Protection (Duty of Care) Regulations 1991 and shall come into force on 1st April 1992.

(2) In these Regulations -

“the 1990 Act” means the Environmental Protection Act 1990;

“transferor” and “transferee” mean respectively, in relation to a transfer of controlled waste by a person who is subject to the duty imposed by section 34(1) of the 1990 Act, the person who in compliance with that section transfers a written description of the waste and the person who receives that description.

Transfer notes

2. -

(1) The transferor and the transferee shall, at the same time as the written description of the waste is transferred, ensure that such a document as is described in paragraph (2) (“a transfer note”) is completed and signed on their behalf.

(2) A transfer note shall -

(a) identify the waste to which it relates and state -

- (i) its quantity and whether on transfer it is loose or in a container;
- (ii) if in a container, the kind of container; and
- (iii) the time and place of transfer;
- (b) give the name and address of the transferor and the transferee;
- (c) state whether or not the transferor is the producer or importer of the waste and, if so, which;
- (d) if the transfer is to a person for authorised transport purposes, specify which of those purposes; and
- (e) state as respects the transferor and the transferee which, if any, of the categories of person shown in column 1 of the following Table describes him and provide any relevant additional information specified in column 2 of the Table. (Please turn over).

Category of Person	Additional Information
An authority which is a waste collection authority for the purposes of Part II of the 1990 Act.	
A person who is the holder of a waste management licence under section 35 of the 1990 Act or of a disposal licence under section 5 of the Control of Pollution Act 1974[2].	If the waste is to be kept, treated or disposed of by that person, the relevant licence number and the name of the licensing authority.
A person to whom section 33(1) of the 1990 Act does not apply by virtue of regulations under subsection (3) of that section.	
A person registered as a carrier of controlled waste under section 2 of the Control of Pollution (Amendment) Act 1989[3].	The name of the waste regulation authority with whom he is registered and his registration number.
A person who is not required to be so registered by virtue of regulations under section 1(3) of that Act.	
A waste disposal authority in Scotland.	

Duty to keep copies of written descriptions of waste and transfer notes

3. The transferor and the transferee shall each keep the written description of the waste and the transfer note or copies thereof for a period of two years from the transfer of the controlled waste.

Duty to furnish documents

4. A person who has been served by a waste regulation authority with a notice in writing specifying or describing any document and requiring its production shall, if the document is one which at that time he is under a duty to keep under regulation 3, furnish the authority with a copy of it at the authority's office specified in the notice and within the period (not being less than 7 days) so specified.

NB – SOME LOCAL AUTHORITIES DO NOT HAVE A WASTE DISPOSAL SERVICE AND SO THE DISPOSAL OF PADS HAVE TO BE RUN IN ACCORDANCE WITH THE LOCAL AUTHORITY POLICY

Communication Policy & Procedure
Crescent Community Care Services Limited

Purpose

- The purpose of this policy is to promote focus on Service User requirements, working efficiency, high productivity and a harmonious working environment by means of effective communications between all members of staff and Service users

Scope

- All Service Users and employees.

Policy

- Communication systems will be set up and maintained to facilitate the flow of information throughout the organization
– This will be determined in differing manners according to the requirement of communication

Service User Information

- Regular meetings of Service Users and families will be organised, supported and the outcomes recorded and communicated to all Service Users and families.
- Memos will be distributed to employees and will be produced on at least an ad-hoc basis and if possible to a known schedule, and all employees are encouraged to read and understand and if they feel that information should be passed over then to contribute material to the memo.
- Service Users are informed before any announced statutory visits.
- Information for or about Service Users received from outside the organisation, e.g. by telephone calls, will be noted on their computerized file and a copy kept on their paper file

Job and Work Related Information

- Job description and/or statement of key tasks.
- Standards of working, including relevant QCF Diploma standards.
- Supervision sessions.
- Appraisal.
- Notices and other written communications.
- Staff Meetings/Team briefings.
- Verbal instructions.
- Custom and practice.

Policies and Procedures

- Crescent Community Care Services Limited is charged with keeping these up to date and for distributing amendments as and when necessary.
- Team leaders/supervisors are responsible for explaining procedures to be followed when operating equipment, completing documentation etc to all other members of staff
- General guidance on health and safety matters will be provided to employees by their manager who is also responsible for supplying information to employees on safe working practices and particular hazards in their own departments.

This will either be by updates on training or self study with documentation sent with the memos.

- Any queries employees may have relating to the work they are required to do should be raised with their supervisor or manager.
- Regular departmental meetings will be held to discuss issues connected with the smooth operation of the department. Arrangements will be made for as many staff to be on duty for such meetings as is possible; staff who attend out of their normal working hours will not be paid for the attendance unless agreed in advance at the manager's discretion.
- Team members are encouraged to make suggestions which they believe may assist in improving efficiency and promoting job satisfaction.
- There will be a Quality Meeting, at least once per quarter in line with our ISO9001 Registration. Any changes will be cascaded down to staff to ensure best practice
- Most areas of work are subject to QCF Diploma standards of working, and where this is the case, these are the standard of performance required. Arrangements will be made for staff in these areas to become knowledgeable about and be trained to these standards.
- Notices will be used where information is not critically important.
- Where the controlled giving and receiving of information is important, a meeting will be used. Meeting times will be issued by managers who will ensure that all members of their teams are read the information in the briefing word for word, and face to face. Feedback is encouraged.
- Overall, where possible, verbal communication is preferred, as it allows a two way communication process, which is far more effective. Where the information is intended to be acted on over a period of time, verbal communication should always be confirmed with a permanent written record.

Quality Related Information

- Information about the overall performance of the establishment, or individual work areas if this level of information is available, will be distributed to all staff reasonably soon after it becomes available. Examples include:
 - Internal audit reports.
 - Responsible Individual reports (where applicable).
 - Periodic reports from the CQC
 - ISO9001 Inspection reports.
 - External audit reports.
- The Quality Policy and Procedure, Quality Meetings Policy and Procedure and the Management Meeting Policy and Procedure each describe the processes by which staff can feed back on the contents of the various quality reporting in the home or agency, and become involved in the continuous improvement programme covering all services provided by the organisation.

Organisation News

- Regular meetings will be held at which the Registered Provider will brief Registered Managers on the organisation's current position and future prospects.
- In turn, Registered Managers have a duty to pass on appropriate information to their team members in the way most suited to their operational requirements, e.g. general meeting, team briefing, etc.
- Suggestions and comments raised by employees should be fully considered and passed to higher authority where appropriate.
- While there is no ideal substitute for providing and receiving information in a face-to-face context, back-up is provided by the organisation in the form of newsletters and other material (see below).
- A memo, distributed to employees, will be produced on an ad-hoc basis, and all employees are encouraged to

contribute material to the memo where necessary.

- Important news items about the organisation which are of immediate interest to all members of staff will be issued via a team briefing note, which will be communicated to all team members verbally within a specified time.
- All information given out by the organisation and its managers should be regarded as commercially sensitive, and only material clearly indicated as non-confidential may be disclosed to members of the public.
- Any queries regarding the information contained in any communications should be raised in the first instance with the departmental manager.
- All employees are reminded that, except for information freely available to members of the general public, no details of the organisation's operations should be disclosed to outsiders without the prior consent of the manager or Registered Provider.

Media Queries

- Any query received from the media must be passed immediately to the nominated manager or the Directors. Employees must not attempt to deal with the query themselves.

Note

- A European directive gives employees in the UK rights to information and consultation. The directive gives employees the right to be informed about the businesses' economic situation and to be informed and consulted about employment prospects and about decisions which may lead to substantial changes in work organisation or contractual relations, including redundancies and transfers. The directive applies to businesses with 50 or more employees. The directive does not apply to businesses with fewer than 50 employees.

Reviewed by Marcus Kerridge-McColl

September 2025

Complaints, Suggestions and Compliments Policy and Procedure
Crescent Community Care Services limited

Purpose

- The Registered Provider operates an effective mechanism for the receipt, recording, investigation and resolution of all complaints, in order to comply with the regulations.
- The arrangements for investigation of complaints are fair and transparent.
- Complaints and suggestions from Service Users or their relatives are a valued source of information regarding the quality of our service, and are a primary source of information regarding possible abuse.
- Care Quality Commission and Local Government Ombudsman guidelines are adhered to.

Scope

- Service Users.
- Relatives.
- Other professionals outside agencies.
- All employees.

Policy

- In all cases complaints and concerns shall be treated seriously in a sensitive and confidential manner.
- Complaints and suggestions must be handled in such a way as to first of all reach a satisfactory outcome with the complainant, and to turn a potentially difficult and damaging problem into a source of quality improvement.
- A copy of this complaints procedure will be given to all Service Users and their representatives at the beginning of the service, and copies will also be made available throughout the service.
- All formal or serious complaints will be investigated by a person not related to the immediate source of the complaint.
- The recording of complaints will not be confined to “serious” or “substantial” complaints. The existence of records for complaints of an apparently minor nature is an indication of the effectiveness of the procedure, the openness of the culture of the organisation and its employees, and their vigilance in the area of abuse.
- Complaints will be recorded on Service Users’ files in order to identify any pattern of complaints relating to an individual, including Care or service provision in order to update and review the Care Planning process.
- Complaints will be recorded centrally in order to identify any pattern of complaint relating to all or a group of Service Users. This record will contain minor complaints in addition to serious complaints, and will be accessible to all members of staff where appropriate, unless this is a safeguarding issue. In order for this to be established, members of staff are to make appropriate entries, in a timely fashion, to Care Planning or risk assessments.
- The central information, with regards to complaints, suggestions and compliments, will be regularly reviewed and analysed. The summary will be regularly considered by the Management Meeting for quality assurance purposes.
- Compliments will be recorded centrally and made available for all parties to read, also on the personnel file of any member of staff individually complimented.
- Employees who are the subject of a complaint should not communicate directly with the complainant unless accompanied by a senior member of staff, unless requested directly to do so by the complainant.
- Where the complaint gives rise to concerns regarding the wellbeing of one or more Service Users, serious consideration must be given to suspension of the person or persons complained about, and an investigation must be initiated immediately in order to identify any risk to the health and welfare of the Service User involved.

Procedure

Standard Arrangements

- There are several distinct levels of dealing with a complaint, and it is important for the speedy and effective resolution that each level is followed.
- The principles applied are:
 - The nearer the person dealing with the complaint is to direct service delivery, the better the likely outcome of the complaint. That person has a better detailed knowledge of the service and can react quickly and appropriately. An exception to this principle will be made in the case of a complaint which alleges abuse, in which case the complaint will be immediately and directly reported to a senior manager. At this point the safeguarding policy should be followed as per local authority advice and the necessary notification made to the CQC.
 - Accepting that personalities can be a factor in complaints, the multiple stages allow this problem to be avoided.
 - The complaints process will only be regarded as “completed” when the complainant or their representative has indicated, in writing if possible, that they are satisfied with the outcome of the complaint procedure.
 - Complaints and suggestions will in all cases be taken seriously, recorded, their practicality/usefulness investigated, and the instigator informed of the decided outcome.
- Appoint a complaints manager within the business that will be responsible for dealing with all complaints. It could be the Registered Manager, a Care Manager or a HR Manager. Within smaller businesses, it is very likely that the service provider him/herself will be the designated complaints manager. Whoever is appointed, they must be in a senior position within the business.
- The Agency must be able to respond to complaints both by email and in writing. Telephone conversations are not sufficient in themselves, as they do not constitute a proper record.



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- It is helpful to have a dedicated email address and inbox for managing the complaints and ensure that someone, preferably the complaints manager, reviews the inbox on a daily basis.
- Update the complainant in writing about your progress during the investigation.
- The complaint procedure must be publicly available. It must be:
 - On your website;
 - Clearly visible in public areas of your registered address;
 - Sent out with all contracts for Care, and;
 - Included within all Service User Guides.
- Investigations and outcomes will be recorded on the complaints form, adding additional sheets as required.
- All employees are warned that written complaints recording rules must be complied with, and those records held where they are freely available to supervisors and managers. Any attempt to conceal a complaint may give rise to formal disciplinary action.
- The complainant will be requested to examine the written records of the complaint and sign to indicate agreement with the outcome.
- In the event of a continued disagreement which cannot be resolved internally, the complainant will be advised to approach an appropriate external authority, such as the CQC, funding authorities such as Social Services or NHS, an independent advocacy service, or the local government Ombudsman.
- For privately funded Service Users a range of advocacy services are available. The Registered Manager should support the Service User to contact an appropriate independent advocate if the Service User shows any signs of being unable to fully make, or further pursue, the complaint.
- The completed complaints form will then be handed to the Registered Manager for permanent filing, centrally and on the Service User file.
- The Management Meeting will periodically (recommended every three months) review all complaints since the previous review in order to identify trends and matters which may have appeared to be relatively minor at the time, but which indicate a deeper problem.
- The services action plan should be updated to include all actions to be taken to resolve any requirements or recommendations made following any investigation.

Written Procedure

- A complaint can be made: by telephone; in writing; by email; or in person. All responses will be made/followed up in writing (preferably email).
- Complaints can be made to:

Crescent Community Care Services Limited
3 Festing Buildings, Highland Road, Southsea, Hampshire, PO4 9DA
02392 738398

info@crescentcare.co.uk
- Complainants must receive an acknowledgment within 24 hours. In this acknowledgment let them know who will be investigating the complaint.
- Set a time limit for the acceptance and investigation of complaints. 12 months is appropriate. Once established you can state that:
 - A complaint must be made no more than 12 months after:
 - The date the event occurred, or if later,

- The date the event came to the notice of the complainant.
- The time limit will not apply if Crescent Community Care Services Limited is satisfied that:
 - The complainant can give a good reason for not making the complaint within that time limit, and;
 - Despite the delay, it is still possible to investigate the complaint effectively and fairly.
- State also how you will deal with anonymous complaints.
- All complaints will be dealt with by a senior person within the Care organisation.
- Complainants will receive (as far as reasonably practical):
 - Assistance to help them understand the complaints procedure; and
 - Advice on where they may obtain such assistance.
- The notice board at your registered address (or other prominent public place) should include a display of advocacy services with contact details. It needs to be regularly reviewed, to ensure that advocacy services contact details are up to date.
- Crescent Community Care Services Limited will only accept complaints from a third party under certain conditions:
 - Either:
 - Where you know that the Service User has consented, either verbally or in writing, or;
 - Where then Service User cannot complain unaided and cannot give consent because they lack capacity within the meaning of the Mental Capacity Act 2005; and
 - The representative is acting in the Service User's best interests – For example, where the matter complained about, if true, would be detrimental to the Service User.
- The procedure must be available, upon request, in other languages and formats. You only need to prepare a special format on request to keep costs low.
- You should state your expected turnaround/resolution time. This should be 28 days, but be realistic advising longer where necessary.
- All complaint investigations should be completed within 6 months at the latest, unless a different time period has been agreed. This should only be done when there is a good reason for it.
- Complainants have the right to refer their complaint to the Local Government Ombudsman if they are unhappy with the outcome of the investigation. The complainant also has the right to alert the Care Quality Commission.
- Suggested wording:

"Once your complaint has been fully dealt with by Crescent Community Care Services Limited, if you are not satisfied with the outcome you can complain to the Local Government Ombudsman (LGO). The LGO provides a free, independent service. You can contact the LGO Advice Team for information and advice, or to register your complaint:

T: 0300 061 0614

E: advice@lgo.org.uk

W: www.lgo.org.uk

The LGO will not usually investigate a complaint until the provider has had an opportunity to respond and resolve matters.

Our service is registered with and regulated by the CQC. The CQC cannot get involved in individual complaints about providers, but is happy to receive information about our services at any time. You can contact the CQC at:

Care Quality Commission (CQC)
National Correspondence
Citygate, Gallowgate
Newcastle upon Tyne NE1 4PA
Tel: 03000 616161
Fax: 03000 616171

03000 616161

Monitoring

- Record the following information on a complaints log:
 - Each complaint received;
 - The subject matter and outcome of each complaint;
 - Details of the reasons for delay where an investigation took longer than the agreed response period agreed; and
 - The date the report of the outcome of the investigation was sent to the complainant.

Annual Reports

- Each provider must prepare an annual report for each year in which it must:
 - Specify the number of complaints received.
 - Specify the number of complaints that the provider decided were well-founded, partly or fully.
 - Specify the number of complaints that the provider has been informed have been referred to other bodies.
 - Give the subject matter of complaints received.
 - Summarise any matters of importance in those complaints themselves or in the way that the complaints were handled.
 - Summarise any matters where action has been or is to be taken to improve services as a consequence of those complaints.
 - A “year” means a period of 12 months ending on 31st March.
 - Providers must ensure that its annual complaint report is available to anyone on request.

Acknowledgement Letters

- The acknowledgement letter should include an offer to discuss the complaint with the complainant at a mutually agreed time, to go over:
 - The manner in which the complaint is to be handled; and
 - The period (response period) within which the investigation of the complaint is likely to be completed.
 - If the complainant does not accept the offer of a discussion, the provider must determine the response period (which should always be within 6 months) and notify the complainant in writing of that period.
 - The acknowledgement can be sent by letter or email.
 - Take account of the complainant’s preferences when communicating with him or her.

Final Response Letter

- The final response letter must include a report giving:
 - A detailed explanation of how the complaint has been considered;
 - The conclusions reached, including any remedial action needed; and confirmation that any action needed has either already been taken or, if not yet taken, the proposed timescale when such action will be completed.
 - The letter must inform complainants of their right to take their complaint to the LGO if they are not happy with the outcome.
 - The final letters should be signed by the “responsible individual” or sent by email in their name.
- If the response is not ready within 6 months, you should:

- Notify the complainant in writing accordingly and explain the reason why; and
- Send the complainant in writing a response in accordance with the above as soon as reasonably practicable after 6 months.

General Information for Staff Managing Complaints

People who can complain

- A complaint can be made by:
 - Someone who receives or has received Care services;
 - Someone who is affected (or likely to be affected) by the action, omission or decision of the provider who is the subject of the complaint; or
 - A representative of either of these, under certain conditions.
 - If a provider is not satisfied that the representative is acting with the Service User's consent or in their best interests, the provider must notify the representative in writing, and state the reason for its decision.

Complaints you do not have to investigate

- Crescent Community Care Services Limited is not required to investigate the following complaints:
 - A complaint by an employee relating to their employment (you should handle this in a different way, for example through your grievance procedure);
 - A complaint that was made in person or by telephone and is resolved to the complainant's satisfaction no later than the next working day after the day the complaint was made; and
 - A complaint that has already been investigated and resolved.
 - In these circumstances, the provider will, as soon as is reasonably practicable, notify the complainant in writing of its decision to not investigate the complaint and the reasons why. It would be best practice to have a standard letter for this purpose.

Duty to co-operate

- If a complaint involves more than one provider/commissioner of services there is a duty on local authorities and the NHS to co-operate and provide a single response. Every provider must work with CCGs or local authorities to provide single response to complaints.

Key Contacts:

Care Quality Commission (CQC)
National Correspondence
Citygate, Gallowgate
Newcastle upon Tyne NE1 4PA
Tel: 03000 616161
Fax: 03000 616171

Social services (for Service Users funded by social services)
0300 555 1386

Portsmouth City Council ASC Department
Tel: 02392 822251

The Local Government Ombudsman
PO Box 4771
Coventry. CV4 0EH
Tel: 0845 602 1983 or 024 7682 1960
Fax: 024 7682 0001
advice@lgo.org.uk

Independent advocacy services

Choices Advocacy,
The Warren Centre Plus,
Warren Crescent,
Shirley Warren,
Southampton SO16 6AY

Reviewed Marcus Kerridge-McColl
December 2025
Confidentiality Policy and Procedure
Crescent Community Care Services Limited

Purpose

- To ensure that all information is collected, recorded, stored, shared and disposed of in the best interests of the Service User, with regard for their human rights and in line with legislation.
- To ensure the Service User is aware of the organisation's confidentiality policy and procedure.

Scope

- All workers and third parties involved in the care of Service Users.

Policy

- The Service User's rights to confidentiality must be safeguarded in accordance with the common law duty of confidentiality and Article 8 of the Human Rights Act 1998. The Care service employee will use information only in the best interests of the Service User and where possible with their express consent. All information, verbal, written, electronic or photographic / video / audio recording is managed in line with the Data Protection Act 1998 and The Access to Health Records Act 1990.

Procedure

Data Collection and Storage

- Staff must ensure that any information collected and stored is limited to that which is essential for the care and safeguarding of the Service User. This relates to personal data and health information essential to establish individual needs and provide staff with the means to plan care and support. Staff must retain information and data securely in locked storage when not in use. Electronic data should be password protected and access limited to essential users only. Photographs and video or sound recordings should not be shared without the express consent of Service Users or their legal advocates.

Disclosure

- Data and information should only be shared with those identified as eligible to access it. This must only be shared for the purposes of ensuring individual needs are met and for the safe delivery of care. So information passed to a care worker may be shared with members of the care team where they are concerned with the care and treatment of the Service User. Staff must be advised of the requirement to confirm the identity and right of access of any third party seeking information about a Service User or The Agency in writing, by telephone or in person before any disclosure is made.
- Consent to disclose information with family members should be obtained from the Service User. There are many good reasons why informal carers should be informed and involved with the professional care of their loved one. If family members wish to pass on their concerns or views to a member of care staff this does not constitute a breach of confidentiality by the care staff.

Consent

- As far as reasonably practicable, written consent to the use of personal information should be gained from each Service User or their advocate. Inform the subject when gaining consent of the specific details of the information/action to be shared and with whom. This should form part of the information provided at the outset of admission to the service.
- If the Service User lacks the mental capacity to consent to information being shared, staff may share information if it is in the Service User's best interests (in accordance with the principles of the Mental Capacity Act 2005). Staff should still clearly explain to the Service User the reasons for the decision to share information and accurately record these.
- Where a Service User has appointed a Lasting Power of Attorney under the Mental Capacity Act 2005 that person should be consulted where information is to be shared in someone's best interests.
- If there are any concerns in these areas you should seek advice from your supervisor.
- Access to data and records must be managed in line with legislation. Permission to access health information may be sought by Service Users or their advocates in writing and access must be managed appropriately. It is good practice to involve Service Users with the collection and recording of personal data as this ensures focus on the needs and rights of each individual. This paragraph should be read in conjunction with the Access to Information Policy and Procedure.
- The use of social media should be avoided unless strict management of information is assured to protect the rights of all individuals in our care. Staff must not discuss confidential information about Service Users in any place where third parties might access it, such as on an outing.
- Staff must ensure that all data and information held pertains only to that individual. Care must be taken to limit recording to identify only the Service User whose record it is, and not to include sensitive identifiable data about others in Care Plans and record entries.
- Confidentiality with respect to a Service User may only be breached if:
 - Where information is required by statute or court order.
 - Information suggests the Service User is at risk of harm to themselves or from others, or others may be at risk of harm from the Service User, and where the passing on of information would be in the person's interest, or the wider public interest.
 - Information, if withheld, could put others at risk; and where the passing on of information would be in the public interest.
 - Where the Service User may have broken the criminal law and where there is justification in the public interest to breach confidentiality, Section 115 of the Crime and Disorder Act 1998 gives a power (but not an automatic duty) to organisations to disclose information to the police 'for the prevention, detection and reduction of crime'. This applies in England and Wales and in Scotland (where the Crime and Disorder Act is amended by the Criminal Justice (Scotland) Act 2003).

Disposal of Confidential Information

- In the event of a Service User's death or leaving the service, records should be removed from circulation and retained in secure archive storage. There is a requirement to retain care and clinical records for not less than 3 years in line with Part 3 section 17 of The Care Homes Regulations 2001. It is good practice to retain records for up to 20 years to meet insurance requirements.
- When confidential information no longer needs to be stored, destruction of paper records must be by shredding to ensure no risk of third parties accessing sensitive data. Destruction of electronic records by deletion must be undertaken by means that ensure no future retrieval is possible.

November 2025

CQC Fundamental Standards Policy and Procedure Crescent Community Care Services

Purpose

- To support and inform compliance with the 11 Fundamental Standards set out in the Health and Social Care Act 2008 (Regulated Activities) Regulations 2014.

Scope

- All activities of the Service.

Policy

- Crescent Community Care Services Ltd will, by following the Quality Compliance Systems Ltd (QCS) compliance management system, meet the requirements of the Regulation, and in particular the 11 Fundamental Standards of care contained in the Regulations.

Relationships between the Regulations, Fundamental Standards, Key Questions and Key Lines of Enquiry.

- The 11 Fundamental Standards are listed in Appendix A. These are the legal requirements placed on all Providers in England.
- While the CQC registration checks and inspections are intended to check the compliance of the Service with the Fundamental Standards, the methodology of doing so is not to inspect against the Standards, but carry out the check by asking 5 Key Questions.
- The 5 Key Questions are expanded into the detail required for inspection through the Key Lines of Enquiry (KLOEs).
- The KLOEs are in turn expanded into Prompts, which guide the CQC inspector in detail during the inspection.
- The complete set of KLOEs flowing from the 5 Key Questions contain all of the requirements set out in the 11 Fundamental Standards in the Regulations.
- Therefore meeting the requirements of the KLOEs also satisfies the need to meet the Fundamental Standards.
- The structure of the cross-referencing between Fundamental Standards, Key Questions and KLOEs is tabulated in Appendix B.

Because the KLOEs resulting from the 5 Key Questions exactly map the Fundamental Standards, and the CQC base inspections on the Key Questions, QCS has structured its compliance support on the 5 Key Questions.

The structure of the QCS compliance support policies is:

- One CQC Fundamental Standards Policy and Procedure (this policy), setting out the overall legal requirements, and how these are met by the QCS compliance management system.
- Five Key Questions Policies, setting out the statement of the standard to be met in relation to that question, listing the KLOEs which support the standard, and the QCS policies which define the activity which supports the KLOEs.
- One Policy for each KLOE, listing the definition of the KLOE, the QCS policies which have a direct relationship to meeting the KLOE (the Primary Policies), and the QCS policies which have a supporting relationship to the KLOE.
- Each KLOE policy also includes an explanation of the policies, documents, activities and direct observational evidence which the CQC inspector may examine in order to determine if the KLOE is being met.
- Each KLOE policy also includes a checklist of evidence to support the Service to check if it is meeting that particular KLOE.

Procedure

QCS recommends that Crescent Community Care Services Ltd carries out training with all employees to make them aware of their responsibilities in meeting the Fundamental Standards. The training should include:

- A brief explanation of the structure of the new requirements as laid out in the Relationships section above should be used to demonstrate that successful compliance with the Standards will be achieved by fully carrying out all of the policies contained in the QCS compliance management system.
- Use of the extensive lists of QCS policies in each KLOE policy to demonstrate to employees, including the Provider and Senior Management, the interdependence of the full suite of QCS policies. Confirm that cherry picking certain policies and ignoring others, for whatever reason, will place Crescent Community Care Services Ltd in danger of being found to be not meeting some of the legal requirements.

Appendix A.

The Health and Social Care Act 2008 (Regulated Activities) Regulations 2014.

Sections 4 to 14, the “Fundamental Standards”

General

3. A registered person must, in so far as they are applicable to each regulated activity, comply with regulations 4 to 14 in carrying on the regulated activities in respect of which they are registered.

SECTION 2

Fundamental standards

Person-centred care

4.—(1) The care and treatment of service users must reflect their needs and preferences.

(2) Paragraph (1) applies—

(a) subject to paragraph (4), and

(b) in so far as it is in accordance with generally accepted professional standards, practices and principles for the care and treatment to reflect such needs and preferences.

(3) The things which a registered person is required to do to comply with paragraph (1) include—

(a) carrying out an assessment of the needs and preferences for care and treatment of the service user;

(b) designing care or treatment with a view to ensuring service users’ welfare;

(c) so far as is reasonably practicable, permitting and encouraging service users to make, or participate in making, decisions relating to their care or treatment to the maximum extent possible;

(d) where the service user is unable to express their preferences because they lack capacity, determining and acting in accordance with the best interests of the service user;

(e) where applicable, providing for the making of reasonable adjustments to meet the service user’s individual needs; and

(f) have sufficient quantities of suitably accessible equipment and medicines to ensure the safety of service users and to meet their assessed needs.

(4) Where Part 4 or 4A of the Mental Health Act 1983(17) applies to a service user, care and treatment must be provided in accordance with the provisions of that Act.

(5) For the purposes of paragraph (3)(d)—



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(a) section 2 of the 2005 Act (people who lack capacity) applies for the purposes of determining whether a service user lacks capacity, and

(b) section 4 of the 2005 Act (best interests) applies as it applies for the purposes of that Act.

Dignity and respect

5.—(1) Service users must be treated with dignity and respect.

(2) The things which a registered person is required to do to comply with paragraph (1) include—

(a) promoting the privacy, autonomy, independence and involvement in the community of the service user; and

(b) having due regard to any protected characteristics (as laid down in section 4 of the Equality Act 2010(18)) of the service user.

Need for consent

6.—(1) Care and treatment must only be provided with, and in accordance with, the consent of the relevant person.

(2) Where the relevant person is unable to give such consent because they lack capacity, the registered person must determine and act in accordance with the best interests of the service user.

(3) Where Part 4 or 4A of the Mental Health Act 1983 applies to a service user, paragraph (1) is subject to the provisions of that Act.

(4) For the purposes of paragraph (2)—

(a) section 2 of the 2005 Act applies for the purposes of determining whether a service user lacks capacity, and

(b) section 4 of the 2005 Act (best interests) applies as it applies for the purposes of that Act.

Safe and appropriate care and treatment

7.—(1) All care and treatment provided to service users must be appropriate and safe.

(2) The things which a registered person is required to do to comply with paragraph (1) include—

(a) taking appropriate steps to mitigate the risks to service users of receiving care or treatment that is inappropriate or unsafe;

(b) taking appropriate steps for the proper and safe management of medicines;

(c) establishing and operating effectively systems designed to assess the risk of, and to prevent, detect and control the spread of, infections, including those that are health care associated; and

(d) where responsibility for the care and treatment of service users is shared with, or transferred to, other persons, working in cooperation with such other persons to ensure that appropriate care planning takes place to ensure the health, safety and welfare of the service users.

(3) For the purposes of paragraph (1), care or treatment will not be appropriate if it is not provided in accordance with generally accepted professional standards, practices and principles or if it includes—

(a) unlawful discrimination against service users, or

(b) control or restraint which is—

(i) unlawful, or

(ii) not necessary to prevent, or not proportionate to, the risk posed if the service user was not subject to control or restraint.

Safeguarding service users from abuse

8.—(1) Service users must not be subject to abuse. (2) Appropriate steps must be taken—

(i) to prevent abuse before it occurs, and

(ii) to respond to any allegation of abuse.

(3) For the purposes of this regulation, “abuse”, in relation to a service user, includes—

(a) sexual abuse,

(b) physical or psychological ill-treatment,

(c) theft, misuse or misappropriation of money or property, or

(d) neglect and acts of omission which cause harm to a service user or place the service user at risk of harm.

Meeting nutritional needs

9.—(1) The nutritional needs of service users must be met.

(2) In this regulation, “nutritional needs” means adequate nutrition to sustain life and good health and includes—

(a) those needs which can be met wholly or in part by the provision of food or drink,

(b) parenteral nutrition and dietary supplements when prescribed by a healthcare professional,

(c) any reasonable requirements arising from a service user’s religious or cultural background, and

(d) where necessary, support to eat and drink.

Cleanliness, safety and suitability of premises and equipment

10.—(1) All premises and equipment used by the registered provider must, in accordance with generally accepted professional standards, practices and principles, be—

(a) safe, clean and secure,

(b) suitable for the purpose for which they are being used, and (c) properly used and maintained.

(2) The registered provider must, in relation to such premises and equipment, maintain standards of hygiene appropriate for the purpose for which they are being used.

(3) For the purposes of this regulation “premises” does not include a service user’s own home

Receiving and acting on complaints

11.—(1) Any complaint received must be appropriately investigated and appropriate action must be taken in response to the complaint.

(2) The registered person must establish and operate effectively a system for identifying, receiving, recording, handling and responding appropriately to complaints, and requests for action to be taken, by service users and other persons in relation to the carrying on of the regulated activity.

(3) The registered person must send to the Commission, when requested to do so, a summary of—

(a) complaints made under such complaints system,

(b) responses made by the registered person to such complaints and any further correspondence with the complainants in relation to such complaints, and

(c) any other relevant information in relation to such complaints as the Commission may request.

Good governance

12.—(1) Systems or processes must be established and operated effectively to ensure compliance with the requirements in this Part.

(2) Such systems or processes must enable the registered person, in particular, to—

(a) assess, monitor and improve the quality of the services provided (including the quality of the experience of service users in receiving those services);

(b) assess, monitor and mitigate the risks relating to the health, safety and welfare of service users and others who may be at risk which arise from the carrying on of the regulated activity;

(c) maintain an accurate, complete and contemporaneous record in respect of each service user, including a record of the care and treatment provided to the service user;

(d) maintain such other records as are appropriate in relation to—

(i) persons employed, and

(ii) the management of the regulated activity;

(e) seek and act on feedback from service users and other persons on the services provided in the carrying on of the regulated activity, for the purposes of continually evaluating and improving such services; and

(f) evaluate and improve their practice in respect of the processing of the information referred to in sub- paragraphs (a) to (e).

Staffing

13.—(1) The registered person must deploy sufficient numbers of suitably qualified, skilled and experienced persons in order to meet the requirements of this Part.

(2) The registered person must ensure that persons employed by the registered provider in the provision of a regulated activity—

(a) receive appropriate support, training, professional development, supervision and appraisal;

(b) are enabled where appropriate, from time to time, to obtain further qualifications appropriate to the work they perform; and

(c) where such persons are healthcare professionals, or social workers or other professionals registered with the Health and Care Professions Council, are enabled to provide evidence to their relevant professional body demonstrating, where it is possible to do so, that they continue to meet the professional standards which are a condition of their ability to practice.

Fit and proper persons employed

14.—(1) Persons employed by the registered person must—

(a) be of good character,

(b) have the qualifications, skills and experience which are necessary for the work to be performed, and (c) be physically and mentally capable, after reasonable adjustments are made, of properly performing tasks which are intrinsic to the work for which they are employed.

(2) Recruitment procedures must be established and operated effectively to ensure that persons employed meet the conditions in paragraph (1).

(3) The information specified in the Schedule and such other information as is appropriate must be available in relation to each such person employed.

(4) Persons employed must be registered with the relevant professional body where such registration is required by, or under, any enactment in relation to—

(a) the work that the person is to perform, or

(b) the title that the person takes or uses.

(5) Appropriate steps must be taken in relation to a person employed by the registered person who no longer meets the criteria in paragraph (1), including—

(a) where the person is a healthcare professional, informing the body responsible for regulation of the health care profession in question, or

(b) where the person is a social worker or other professional registered with the Health and Care Professions Council, informing the Council.

Reviewed by Marcus Kerridge McColl
November 2025

Cross Contamination between Care and Food Preparation Policy and Procedure Crescent Community Care Services Limited

Purpose

- To comply with statutes, regulations and quality standards. §
- To prevent cross-contamination.

Scope

- All workers.

Policy

- Clothing and personal hygiene will be managed in order to minimise the risk of cross-contamination between Care and food preparation activities.

Procedure

- Care staff who are carrying out activities which may lead to contamination of food must wear barrier clothing (e.g. disposable apron) when carrying out care, and that clothing must be removed before entering the food preparation area.
- Care staff may enter food preparation areas for food preparation purposes only.

Reviewed by Marcus Kerridge McColl
December 2025

Data Security Breach Policy & Procedure Crescent Community Care Services Limited

Purpose

- To identify potential data security breach risks, specify safeguards, and state information handling in the event of a breach.
- To meet the seventh data protection principle of the Information Commissioner's Office (ICO), which is:
 - "Appropriate technical and organisational measures shall be taken against unauthorised or unlawful processing of personal data and against accidental loss or destruction of, or damage to, personal data."

Scope

- All data relating to Crescent Community Care Services Limited, its Service Users and employees held within the organisation or its associates, however stored.
- For obligations under the Data Protection Act also refer also Access to Information Policy and Procedure.

Policy

- Crescent Community Care Services Limited, as an organisation which processes personal data, must take appropriate measures against unauthorised or unlawful processing and against accidental loss, destruction of or damage to personal data.
- A data security breach can happen for a number of reasons:
 - Loss or theft of data or equipment on which data is stored;
 - Inappropriate access controls allowing unauthorised use;
 - Equipment failure;
 - Human error;
 - Unforeseen circumstances such as a fire or flood;
 - Hacking attack;
 - 'Blagging' offences where information is obtained by deceiving the organisation who holds it.

Procedure

- If breach has occurred, there are four important elements to any breach management plan:
 - Containment and recovery;
 - Assessment of ongoing risk;
 - Notification of breach;
 - Evaluation and response.

Containment and Recovery

- Data security breaches will require not just an initial response to investigate and contain the situation but also a recovery plan including, where necessary, damage limitation. This will often involve input from specialists across the business such as IT, HR and legal and in some cases contact with external stakeholders and suppliers. Consider the following:
 - Decide on who should take the lead on investigating the breach and ensure they have the appropriate resources.
 - Establish who needs to be made aware of the breach and inform them of what they are expected to do to assist in the containment exercise. This could be isolating or closing a compromised section of the network, finding a lost piece of equipment or simply changing the access codes at the front door.
 - Establish whether there is anything you can do to recover any losses and limit the damage the breach can cause. As well as the physical recovery of equipment, this could involve the use of backup media to restore lost or damaged data or ensuring that staff recognise when someone tries to use stolen data to access accounts.
 - Where appropriate, inform the police.

Assessing the Risks

- Some data security breaches will not lead to risks beyond possible inconvenience to those who need the data to do their job. An example might be where a laptop is irreparably damaged but its files were backed up and can be recovered, albeit at some cost to the business. While these types of incidents can still have significant consequences the risks are very different from those posed by, for example, the theft of an employee database, the data on which may be used to commit identity fraud. Before deciding on what steps are necessary, further to immediate containment,



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assess the risks which may be associated with the breach. Perhaps most important is an assessment of potential adverse consequences for individuals, how serious or substantial these are and how likely they are to happen.

- The following points are also likely to be helpful in making this assessment:
 - What type of data is involved?
 - How sensitive is it? Remember that some data is sensitive because of its very personal nature (health records) while other data types are sensitive because of what might happen if it is misused (bank account details).
 - If data has been lost or stolen, are there any protections in place such as encryption?
 - What has happened to the data? If data has been stolen, it could be used for purposes which are harmful to the individuals to whom the data relate; if it has been damaged, this poses a different type and level of risk.
 - Regardless of what has happened to the data, what could the data tell a third party about the individual? Sensitive data could mean very little to an opportunistic laptop thief while the loss of apparently trivial snippets of information could help a determined fraudster build up a detailed picture of other people.
 - How many individuals' personal data are affected by the breach? It is not necessarily the case that the bigger risks will accrue from the loss of large amounts of data but is certainly an important determining factor in the overall risk assessment
 - Who are the individuals whose data has been breached? Whether they are staff, customers, clients or suppliers, for example, will to some extent determine the level of risk posed by the breach and, therefore, your actions in attempting to mitigate those risks.
 - What harm can come to those individuals? Are there risks to physical safety or reputation, of financial loss or a combination of these and other aspects of their life?
 - Are there wider consequences to consider such as a risk to public health or loss of public confidence in an important service you provide?
 - If individuals' bank details have been lost, consider contacting the banks themselves for advice on anything they can do to help you prevent fraudulent use.

Notification of Breaches

- Informing people and organisations that you have experienced a data security breach can be an important element in your breach management strategy. However, informing people about a breach is not an end in itself.
- Notification should have a clear purpose, whether this is to enable individuals who may have been affected to take steps to protect themselves or to allow the appropriate regulatory bodies to perform their functions, provide advice and deal with complaints.
- From 26 May 2011 certain organisations (service providers) have a requirement to notify the Information Commissioner (ICO), and in some cases individuals themselves, of personal data security breaches.
- For more information about the specific breach notification requirements for service providers see ICO on the internet
- Answering the following questions will assist other types of organisations in deciding whether to notify:
 - Are there any legal or contractual requirements? Service providers have an obligation to notify the Commissioner in certain circumstances. Health and Social Care providers will have a legal responsibility to notify their Regulator of breaches, and may have a contractual obligation to notify commissioners of services, such as Social Services or NHS.
 - Can notification help you meet your security obligations with regard to the seventh data protection principle? This is "Appropriate technical and organisational measures shall be taken against unauthorised or unlawful processing of personal data and against accidental loss or destruction of, or damage to, personal data."
 - Can notification help the individual? Bearing in mind the potential effects of the breach, could individuals act on the information you provide to mitigate risks, for example by cancelling a credit card or changing a password?



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- If a large number of people are affected, or there are very serious consequences, you should inform the ICO.
- Consider how notification can be made appropriate for particular groups of individuals, for example, if you are notifying children or vulnerable adults.
- Have you considered the dangers of 'over notifying'. Not every incident will warrant notification and notifying a whole customer base of an issue affecting only one customer may well cause disproportionate enquiries and work.
- You also need to consider who to notify, what you are going to tell them and how you are going to communicate the message. This will depend to a large extent on the nature of the breach but the following points may be relevant to your decision:
 - Make sure you notify the appropriate regulatory body. A sector specific regulator may require you to notify them of any type of breach but the ICO should only be notified when the breach involves personal data.
 - There are a number of different ways to notify those affected so consider using the most appropriate one. Always bear in mind the security of the medium as well as the urgency of the situation.
 - Your notification should at the very least include a description of how and when the breach occurred and what data was involved. Include details of what you have already done to respond to the risks posed by the breach.
 - When notifying individuals give specific and clear advice on the steps they can take to protect themselves and also what you are willing to do to help them.
 - Provide a way in which they can contact you for further information or to ask you questions about what has occurred – this could be a helpline number or a web page, for example.
 - When notifying the ICO you should also include details of the security measures in place such as encryption and, where appropriate, details of the security procedures you had in place at the time the breach occurred. You should also inform us if the media are aware of the breach so that we can manage any increase in enquiries from the public. When informing the media, it is useful to inform them whether you have contacted the ICO and what action is being taken. ICO will not normally tell the media or other third parties about a breach notified to us, but we may advise you to do so.
- The ICO has produced guidance for organisations on the information we expect to receive as part of a breach notification and on what organisations can expect from us on receipt of their notification.
 - This guidance is available on their website
 - You might also need to consider notifying third parties such as the police, insurers, professional bodies, bank or credit card companies who can assist in reducing the risk of financial loss to individuals, and trade unions.

Evaluation and Response

- It is important not only to investigate the causes of the breach but also to evaluate the effectiveness of your response to it. Clearly, if the breach was caused, even in part, by systemic and ongoing problems, then simply containing the breach and continuing 'business as usual' is not acceptable; similarly, if your response was hampered by inadequate policies or a lack of a clear allocation of responsibility then it is important to review and update these policies and outline responsibility in the light of experience.
- You may find that existing procedures could lead to another breach and you will need to identify where improvements can be made.
- The following points will assist you:
 - Make sure you know what personal data is held and where and how it is stored. Dealing with a data security breach is much easier if you know which data are involved. Your notification with the Information Commissioner will be a useful starting point.
 - Establish where the biggest risks lie. For example, how much sensitive personal data do you hold? Do you store data across the business or is it concentrated in one location?
 - Risks will arise when sharing with or disclosing to others. You should make sure not only that the method of

transmission is secure but also that you only share or disclose the minimum amount of data necessary. By doing this, even if a breach occurs, the risks are reduced.

- Identify weak points in your existing security measures such as the use of portable storage devices or access to public networks.
- Monitor staff awareness of security issues and look to fill any gaps through training or tailored advice.
- Consider whether you need to establish a group of technical and nontechnical staff who discuss 'what if' scenarios – this would highlight risks and weaknesses as well as giving staff at different levels the opportunity to suggest solutions.
- If you have completed the Business Continuity Plan for dealing with serious incidents, consider implementing a similar plan for data security breaches, or incorporating security breaches into the overall Business Continuity Plan. Breach of data security could in some circumstances be serious enough to endanger the business.
- It is recommended that at the very least you identify a group of people responsible for reacting to reported breaches of security.

Other information

- Additional guidance is also available if you need further information on data security breaches

Reviewed Marcus Kerridge-McColl

September 2025

DBS/Disclosure Policy and Procedure
Crescent Community Care Services Limited

Scope

- All workers who have contact with Service Users, in whatever capacity.

Policy

General principles

- As an organisation we use the DBS service to help assess the suitability of applicants for positions of trust. We comply fully with the DBS Code of Practice regarding the correct handling, use, storage, retention and disposal of Disclosures and information. We also comply fully with its obligations under the Data Protection Act and other relevant legislation pertaining to the safe handling, use, storage, retention and disposal of Disclosure information and have a written policy on these matters. This is available to those who wish to see it on request.
- Employees covered by the Police Act 1997 are defined as "Care Workers". The official definition is:
- Definition of a care worker, from the Health and Social Care Act 2008. Care Worker means:
 - An individual who is or has been employed in a position which is such as to enable them to have regular contact in the course of their duties with adults to whom accommodation is provided at a care home or agency;
 - An individual who is or has been employed in a position which is such as to enable them to have regular contact in the course of their duties with adults to whom prescribed services are provided by an independent hospital, an independent clinic, an independent medical agency or a national health service body;
 - An individual who is or has been employed in a position which is concerned with the provision of personal Care in their own homes for persons who by reason of illness, infirmity or disability are unable to provide it for themselves without assistance.
- Care Workers must not be employed unless they have been confirmed as not listed on the DBS Register.
- Care Workers must not have their employment confirmed until a satisfactory Disclosure is received.
- Care Workers must not be employed unless a check of the Nursing and Midwifery Council Register in the case of Registered Nurses proves positive.

- While employees' DBS checks are in progress, the employee will be supervised at all times, and will not have unsupervised access to Service Users. **BEWARE: Some CQC areas insist that new employees are not employed in contact with Service Users until the check has been received and assessed.**
- Disclosure information is never kept on an applicant's personnel file and is always kept separately and securely, in lockable, non-portable, storage containers with access strictly controlled and limited to those who are entitled to see it as part of their duties.
- In accordance with section 124 of the Police Act 1997, Disclosure information is only passed to those who are authorised to receive it in the course of their duties. We maintain a record of all those to whom Disclosures or Disclosure information has been revealed and we recognise that it is a criminal offence to pass this information to anyone who is not entitled to receive it.
- Disclosure information is only used for the specific purpose for which it was requested and for which the applicant's full consent has been given.
- Once a recruitment (or other relevant) decision has been made, we do not keep Disclosure information for any longer than is absolutely necessary. This is generally for a period of up to six months, or immediately following the next **CQC** inspection, whichever is the later, to allow for the consideration and resolution of any disputes or complaints. If, in very exceptional circumstances, it is considered necessary to keep Disclosure information for longer than six-months, we will consult the DBS about this and will give full consideration to the Data Protection and Human Rights of the individual subject before doing so. Throughout this time, the usual conditions regarding safe storage and strictly controlled access will prevail.
- Once the retention period has elapsed, we will ensure that any Disclosure information is immediately suitably destroyed by secure means, i.e. by shredding, pulping or burning. While awaiting destruction, Disclosure information will not be kept in any insecure receptacle (e.g. waste bin or confidential waste sack). We will not keep any photocopy or other image of the Disclosure or any copy or representation of the contents of a Disclosure. However, notwithstanding the above, we may keep a record of the date of issue of a Disclosure, the name of the subject, the type of Disclosure requested, the position for which the Disclosure was requested, the unique reference number of the Disclosure and the details of the recruitment decision taken.
- Disclosure information may not be passed on to a third party, for instance as part of a reference to a subsequent employer.

Procedure

- Employees must register with the DBS direct, or through an "umbrella body". Contact the DBS for details.
- All employees offered a position:
 - Must not be offered a post until a satisfactory DBS check has been received, either together with the Disclosure or the DBS First procedure;
 - If a DBS First check has been received and is satisfactory, the employee may be offered the post subject to confirmation which is subject to receipt of a satisfactory Disclosure DBS Check.
 - The check will be an Enhanced level for those employees having direct access to Service Users, and Standard level for those who do not have direct access. Best practice dictates that Enhanced Level checks are used in all instances, due to the practical difficulties of ensuring that a person subject to only a Standard check never has unsupervised access to Service Users.
 - Be the subject of a Disclosure Application Form to the DBS;
 - Be informed that will they be supervised at all times, and will not have unsupervised access to Service Users. (Or, in those areas where the Care Quality Commission so insist, informed that they may not be employed where they will be in contact with Service Users until the check has been received);
 - Have a member of staff, who has satisfactorily completed the DBS check process attached to them on each shift as a supervisor, and who ensures that the unchecked employee does not have lone one-to-one contact with any vulnerable person, until the check process has been satisfactorily completed.
- On receipt of the Disclosure, the Registered Manager, in consultation with the Registered Provider if necessary, must assess the information and make a decision as to whether or not to confirm the employment. Refer to the section on employment of offenders, below.



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- Note 1: The Registered Provider must always be consulted in the event of a conviction or relevant caution being revealed on a Disclosure, and where it may be proposed to employ the applicant. The decision to employ must rest with the Registered Provider and be explicitly recorded as such.
- Note 2: The Protection of Freedoms Act 2012 makes provision in the future (date to be announced) for Disclosure to be sent only to the applicant and not the proposed employer, thereby requiring the applicant to supply the document to the Employer. The Employer will have no right to receive the Disclosure directly.
- Other documents and policies which refer, and which must be followed in order to meet the standard, are as follows:
 - DBS Policies and documents attached to this policy;
 - Recruitment – Individual Candidate Application;
 - Induction Policy and Procedure.
- The Registered Provider and Registered Manager will be DBS Checked at the Enhanced Disclosure level via the Care Quality Commission.
- On the 17th June 2013 the DBS launched the “Update Service”. By paying an annual subscription applicants can register their Disclosure online and give permission to future employers to check this information online rather than applying for a new Disclosure.
- With permission from the applicant an employer can carry out a free status check providing the check required is of the same level and type.
- Given that the status check does not reveal any new information regarding the applicant since the original Disclosure was registered, the Disclosure can be accepted and used.
- The Update Service allows DBS Disclosures to be portable from employer to employer as well as allowing an existing Disclosure to be checked, with permission, as frequently as an employer deems appropriate.
- More information on the Update Service can be found at: <https://www.gov.uk/government/publications/dbs-update-service-employer-guide>